PEER REVIEW HISTORY

BMJ Open publishes all reviews undertaken for accepted manuscripts. Reviewers are asked to complete a checklist review form (**http://bmjopen.bmj.com/site/about/resources/checklist.pdf**) and are provided with free text boxes to elaborate on their assessment. These free text comments are reproduced below.

ARTICLE DETAILS

TITLE (PROVISIONAL)	Prevalence of Lateral Violence in Nurse Workplace: A Systematic Review and Meta-analysis
AUTHORS	Zhang, Yue; Cai, Jianzheng; Yin, Rulan; Qin, Shuwen; Wang, Haifang; Shi, Xiaoqing; Mao, Lifen

VERSION 1 – REVIEW

REVIEWER	Serafin, Lena Medical University of Warsaw, Department of Clinical Nursing
REVIEW RETURNED	22-Sep-2021

GENERAL COMMENTS	Dear Authors, congratulation for your work. You touched on the very important issue of lateral violence which requires data synthesis. Please find some recommendations for you; some areas need clarification to make the paper robust for an international audience. Additionally, please add as a supplementary file the PRISMA checklist. Introduction I recommend the authors to situate the topic of lateral violence in the whole issue of violence in the workplace. There is also need to rationale more why this type of violence is particularly important in the nursing environment and what causes such behavior. I strongly suggest to report also LV impact on the quality of care. To make the theme relevant also for its consequences at the patient's level. Methods There is need to add the information about the year of PRISMA guidelines publication which has been used in the study. Please describe the reasons for excluding articles after full text reading. There is no information in Flow diagram regarding the number of papers obtained from screening references. Also, to be transparent in the presentation of your data, please add information on the number of articles retrieved from each search database. Additionally, please add the information about sources of review (peer-reviewed journals? Dissertation?). Was the grey literature screened? How many papers have been excluded after quality assessment and why? There is need to escribe quality items which was considering. Why did you include to the analysis the studies asseses as a low-quality studies? Did you assess methodological quality of analyzed studies? Please specify if you have registered this review.

	Results The narrative presentation of the studies included in the analysis lacks the characteristics of the studied sample in terms of the specificity of the workplace or sociodemographic data. Disscusion Few non-Asian studies were included in the study. In particular, the analyzes carried out in Europe are missing. In the study, this thread should be further developed, and the authors should discuss the results in terms of such a small number of results of the searched studies from Europe, or the lack of Australian, African reports. Workplace violence is a widely recognized issue globally. Please, discuss implications of the results for practice. Conclusion There is lack of information about the need of pay more attention to the cultural differences of inter-nurse LV

REVIEWER	Martinez Jarreta, Begoña University of Zaragoza, Occupational and Forensic Medicine
REVIEW RETURNED	17-Nov-2021

GENERAL COMMENTS	Lateral violence among nurses is an emerging topic in scientific research and adequate characterization of it is needed. The authors present a systematic review and meta-analysis conducted on the basis of the Preferred Reporting Items for Systemic Reviews and Meta-Analyses (PRISMA) guidelines.This is the first Systematic Review and Metanalyses on this topic and deserved being published. The study design is devoted to give an appropriate to answer a research question that remains unclear despite the number of studies carried up to date (in part because the frequent inconsistency of most of them). Recommendation is to accept without amends with the exception of a particular mention and a short discussion which is still needed on the poor representation of European studies amenable to be included in the systematic review. Also on the interpretation of this fact. Furthermore an Asian-no Asian comparison has been used by authors. Although they have discussed on this, however the non Asian group is so heterogeneous and the data from Europe so limited (a unique included study) that some extra consideration on this is needed (on the European lack of studies in general and concerning to to this study outcomes in particular). The heterogeneity among European and non Asian has been pointed out in some way but the lack of studies to be included should be emphasized.

VERSION 1 – AUTHOR RESPONSE

For Reviewer 1：

Dear Dr. Lena Serafin,

We feel very honored to your affirmation and thanks for your suggestions very much. Your comments are very important to our article. We have responded to your comments one by one:

1. Add as a supplementary file the PRISMA checklist.

Response: We have added the PRISMA checklist to supplementary file, named PRISMA checklist.

2. Introduction

I recommend the authors to situate the topic of lateral violence in the whole issue of violence in the workplace. There is also need to rationale more why this type of violence is particularly important in the nursing environment and what causes such behavior.

I strongly suggest to report also LV impact on the quality of care. To make the theme relevant also for its consequences at the patient's level.

Response: We have modified the introduction part, and the detailed content is shown in red font in the relevant section. Please see page 3-4, Introduction.

3. Methods

(1) There is need to add the information about the year of PRISMA guidelines publication which has been used in the study.

Response: Thanks for your reminder, we have added the year of PRISMA guidelines (2020) to the article.

(2) Please describe the reasons for excluding articles after full text reading. There is no information in Flow diagram regarding the number of papers obtained from screening references. Also, to be transparent in the presentation of your data, please add information on the number of articles retrieved from each search database.

Response: We have modified the flow diagram. Added information about the screening references for reading the full text and the number of articles retrieved from each search database to the flow diagram.

(3) Additionally, please add the information about sources of review (peer-reviewed journals? Dissertation?). Was the grey literature screened?

Response: Thanks for your suggestions very much. As we took the current global situation of LV prevalence into account, so the published studies were focused on, no grey literature was screened. This is also our limitation, which we mentioned in the discussion. Please see page 16, row 11-17.

(4) How many papers have been excluded after quality assessment and why? There is need to escribe quality items which was considering. Why did you include to the analysis the studies assess as a low-quality studies?

Response: We didn't exclude any low-quality study. The study quality mainly focuses on sample representativeness and size, comparability between respondents and non-respondents, Assays of assays and statistical quality. Given the view that the primary purpose of our article was to understand the LV prevalence in the world, low-quality studies were included in the article after comprehensive consideration. In addition, we have taken into the quality problem you mentioned, so after the study quality assessment, we divided the study quality into high-quality and low-quality for subgroup analysis, and the results showed that there was no difference between the two groups.

(5) Did you assess methodological quality of analyzed studies?

Response: The M-NOS scale we used contains methodological quality assessment, which assesses quality in several domains: sample representativeness and size, comparability between respondents and non-respondents, ascertainment of adherence symptoms, and statistical quality.

(6) Please specify if you have registered this review.

Response: Thanks for your suggestions very much. We didn't have registered this review.

4. Results

(1) The narrative presentation of the studies included in the analysis lacks the characteristics of the studied sample in terms of the specificity of the workplace or sociodemographic data.
Response: Most of the included studies were region-wide Internet surveys, lacking descriptions of workplace characteristics, which made it impossible to collect specific data such as nurse level and workplace. Hence, the specificity data of the workplace included country and sociodemographic data included gender, age. According to your suggestion, we have added nursing experience data to consider its impact on LV prevalence, as showed in Table 1 on page 7-8 of the manuscript.
5. Discussion
(1) Few non-Asian studies were included in the study. In particular, the analyzes carried out in Europe are missing. In the study, this thread should be further developed, and the authors should discuss the results in terms of such a small number of results of the searched studies from Europe, or the lack of Australian, African reports. Workplace violence is a widely recognized issue globally.
Response: Thanks for your suggestions very much. We focused on LV among nurses in the nursing workplace worldwide, so only studies where the abuser identified as colleagues were included, resulting in the exclusion of studies that did not meet this criterion. Therefore, the low number of non-Asian studies suggests that other countries do not pay enough attention to the sources of abusers. As the author suggests, LV is a global problem, so non-Asian countries need to increase research in this area in order to understand the global situation. In addition, we recorded the region of all studies that met the inclusion criteria. However, due to the few studies and complex distribution areas, it can't be analyzed. We can only subdivide the region subgroup to analysis. Meanwhile, we have modified it in the discussion part to explain the thread of lack of studies.
(2) Please, discuss implications of the results for practice.
Response: We have added this part to the manuscript. Please see page 14 in red.
6. Conclusion
(1) There is lack of information about the need of pay more attention to the cultural differences of inter-nurse LV.
Response: We have added the information that emphasizes the cultural differences of nurses LV to the manuscript. Please see page 15, Conclusion.

For Reviewer 2：
Dear Prof. Begoña Martinez Jarreta,
We feel very honored to your affirmation and thanks for your suggestions very much. We have responded to your comments as follows:
(1) A particular mention and a short discussion is needed on the poor representation of European studies amenable to be included in the systematic review.
Also on the interpretation of this fact. Further more an Asian-no Asian comparison has been used by authors. Although they have discussed on this, however the non Asian group is so heterogeneous and the data from Europe so limited (a unique included study) that some extra consideration on this is needed (on the European lack of studies in general and concerning to this study outcomes in particular). The heterogeneity among European and non Asian has been pointed out in some way but the lack of studies to be included should be emphasized.
Response: In the discussion section of the Asian and non-Asian subgroups, country subgroup analysis was added to explore differences in LV prevalence among different countries. Please see page 11, Table 2. In addition, the lack of studies inclusion was highlighted in the discussion, as showed on page 13, row 8-14.

VERSION 2 – REVIEW

REVIEWER	Serafin, Lena Medical University of Warsaw, Department of Clinical Nursing
REVIEW RETURNED	28-Dec-2021

GENERAL COMMENTS	Generally, the authors have addressed all my comments, and I am satisfied with the changes that have been made. In my opinion, the manuscript is a valuable and well-developed material that allows for a deeper understanding of lateral violence. Congratulations to the authors of a good study.